# A/PROF CHAMARA BASNAYAKE
**MBBS, PhD, FRACP**
**Gastroenterologist & Endoscopist**
**PN 421926PK**

FOCUS GASTROENTEROLOGY


29 April 2026

Dr Kerryn Gijsbers
East Brunswick Medical Centre

Dear Dr Gijsbers,

**Re: Ms Millie Collodetti**
**DOB: 11/10/2005 Phone: 0491 013 785**
**Address: 9 Alister Street, FITZROY NORTH, VIC, 3068**
**Email: millie@collodetti.com**
**Medicare: 3310160997 / 6**

**Summary**

- **Normal biopsies of the small intestine and colon, excluding coeliac disease, Helicobacter pylori, inflammatory bowel disease, and malignancy.**
- **Internal haemorrhoids identified as the cause of previous haematochezia.**
- **Diagnosis of irritable bowel syndrome mediated by visceral hypersensitivity and pelvic floor dysfunction.**
- **Commenced nortriptyline titration for symptom management.**
- **No action required by you.**

I had the pleasure of conducting a review with Millie following her recent gastroscopy and colonoscopy. You will be pleased to note that all histological evaluations were unremarkable. Biopsies of the small intestine excluded coeliac disease, and gastric biopsies were negative for Helicobacter pylori. Furthermore, there was no evidence of Crohn's disease, ulcerative colitis, microscopic colitis, or malignancy. The colonoscopy did identify internal haemorrhoids, which accounts for her previous history of rectal bleeding.

Millie continues to experience symptoms consistent with irritable bowel syndrome, specifically abdominal pain, nausea, gaseousness, and loose stools. She has noted a partial improvement in her bowel habits and the sensation of incomplete evacuation by incorporating kiwi fruit, guar gum, and the use of a footstool to optimise her defaecation posture. We discussed the underlying pathophysiology of her symptoms, which I believe are primarily driven by visceral hypersensitivity and a degree of pelvic floor dysfunction.

**Impression and Plan**

While we discussed several therapeutic avenues, including gut-directed hypnotherapy and pelvic floor physiotherapy, Millie has elected to proceed with pharmacological management at this stage. I have commenced her on a low dose of nortriptyline to address her visceral hypersensitivity and nausea. She will start with 10 mg at night, titrating up to 20 mg after several days if she tolerates the medication without significant side effects, such as sedation or xerostomia.

I have explained that the goal is to modulate the neural pathways of the gastrointestinal tract and that the full benefit may not be apparent for four to six weeks. I have advised Millie to take the medication in the evening

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to mitigate any daytime grogginess. I intend to review Millie in eight weeks to assess her clinical response. If she experiences significant improvement, we will likely maintain the therapy for approximately twelve months. Should her symptoms remain refractory, we will reconsider the involvement of a pelvic floor physiotherapist or a gut-directed hypnotherapist.

Yours sincerely,

**A/Prof Chamara Basnayake**
Gastroenterologist

**Investigation Results:**

COLLODETTI, MILLIE ANNE MIMA : SVP 925244

DOB: 11/10/2005 Age: 20 years

9 Alister St, Fitzroy North 3068

Results from 11/04/2026 ..... Episode# 26P258059 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009386 **CLINICAL NOTES** Gas = N. Duo = N. Oesophagus = N. Random colon = N. IBS. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophagus". Three mucosal fragments measuring 1-5 mm. A1. 2. Specimen labelled "Gastric". Three mucosal fragments measuring 1-4 mm. A1. 3. Specimen labelled "Duodenum". Three mucosal fragments measuring 3-5 mm. A1. 4. Specimen labelled "Random Colon". Five mucosal fragments measuring 1-7 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The sections show oesophageal mucosa with stratified squamous epithelium. There is no basal cell hyperplasia, acute inflammation or ulceration. There is no metaplasia, dysplasia or malignancy. 2. The sections show transitional gastric mucosa with corkscrew foveolar architecture. There is a mild increase in chronic inflammatory cells within the lamina propria and vertically oriented smooth muscle splaying between glands. No active inflammation, atrophy or fibrosis is seen and Helicobacter pylori-like organisms are not identified. There is no intestinal metaplasia, dysplasia or malignancy. 3. The sections show duodenal mucosa with Brunner's glands and maintained villous to crypt ratios. There is a normal complement of chronic inflammatory cells within the lamina propria. There is no increase in intraepithelial lymphocytes at villous tips. No active inflammation, granulomata or organisms are identified. There is no dysplasia or malignancy. 4. The sections show colonic mucosa with maintained crypt architecture. There is a normal complement of chronic inflammatory cells in the lamina propria. The subepithelial collagen plate is of normal thickness. No active inflammation or granulomata are seen. There is no dysplasia or malignancy. **PATHOLOGIST'S OPINION** **1. Oesophageal biopsy: Normal squamous mucosa. 2. Gastric biopsy: Changes of reactive gastropathy. 3. Duodenal biopsy: Normal duodenal mucosa. 4. Random colon biopsy: Normal colonic mucosa.** Pathologist: Dr Rohini Joshi

All appointments and enquires
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Endoscopy locations
**Epworth Freemasons Day Proc. Centre**
320 Victoria Pde, East Melbourne, 3002
**Hobson Healthcare Sydenham**
566 Melton Hwy, Sydenham, 3037
**St Vincent's Private Hospital**
59 Victoria Pde, Fitzroy, 3065